Drug Abuse: Problem, Prevention and management

Study Material

UNIT IV (Drug abuse: problem, prevention and management)

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Syllabus

UNIT IV (DRUG ABUSE: PROBLEM, PREVENTION AND MANAGEMENT)

1. Problem of drug abuse
   Meaning and concept; types of drugs often misused; antibiotics, steroids and their misuse with suitable examples; habit forming drugs and their effects; drug addiction; drug tolerance and withdrawal symptoms; various signs and symptoms of drug abuse; vulnerable groups (age, gender and socio-economic status)
2. Prevention and management of drug abuse
   Medical treatment through deaddiction center, support of family, media and school education; Government policies, programs and laws to prevent drug abuse.

Instructions to the examiners

1. Unit IV of the paper dealing with Drug abuse would be of 20 marks comprising of twenty multiple choice questions carrying one mark each having four options a, b, c, d with one correct and three incorrect alternatives.
2. The question paper shall be set strictly according to the syllabus uniformly
3. All the questions are to be attempted.

Note: The teaching hours for the Unit IV will be 10 hours

CONTENTS

Chapter 1- DRUG ABUSE	**1-14**
• Meaning and concept	1
• Types of drugs often misused; antibiotics, steroids and their misuse with suitable examples	2
• Habit forming drugs and their effects	6
• Vulnerable groups (age, gender and socio-economic status)	7
• Drug Addiction	9
• Withdrawal from drugs	11
Chapter 2- PREVENTION AND MANAGEMENT OF DRUG ABUSE	15-37
• Drug addiction centre	17
• Support of family, media and school education	23
• Government policies, programs and laws to prevent drug abuse	29

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CHAPTER 1
DRUG ABUSE

MEANING AND CONCEPT

What is a drug?

First of all, let us try to understand the meaning of the word drug. A drug is a substance that can be taken into the human body and, once taken, alters some processes within the body. Drugs can be used in the diagnosis, prevention, or treatment of a disease. Some drugs are used to kill or stop the growth of microorganisms such as bacteria, viruses, fungi etc. and help the body recover from infections while some assist in terminating headaches, pain and also more serious ailments such as cancer. Some examples of drugs are penicillin's, paracetamol, aspirin etc.

What is drug abuse?

Now the question is 'what is drug abuse?' In simple words it is 'misuse of any chemical or drug for a purpose other than the one for which it is indicated'. The main purpose of using these drugs is to get pleasurable effect on the brain. Drugs like cannabis, cocaine, heroin etc. or some prescribed medicines like morphine come under this category and are misused. Use of certain medicines available over the counter like pain killers or cough syrups in amounts or by methods which are harmful to an individual or others is also a form of drug abuse. In general, we may say that drug abuse is illegal non-medical use of some substances, most of which are drugs. These substances tend to alter one's mental state in such a way which is not acceptable to society and is considered illegal.

REASONS OF DRUG ABUSE

There could be many reasons due to which a person might start drug abuse. Some of these are listed below.

1. **Experimental use of a recreational drug in social situation**
   At times a person tries a particular drug for fun purpose with friends or relatives and starts liking it.
2. **Peer pressure**
   Sometimes a person starts taking a drug due to pressure from his friends.
3. **Exposure to medications**

A person might get used to a particular drug through medications prescribed for a certain health problem and becomes so dependent on that specific drug that he continues using the same even after the expiry of the prescribed period. Some people come across drugs through medications suggested by friends or relatives. These suggestions can come either through word of mouth or self-medication for getting relief from a malady.

TYPES OF DRUGS OFTEN MISUSED

The drugs may be classified on the basis of chemical structure, pharmacological effects or perceived medical value (risk involved) as shown below.

Chemical classification

Alcohol
Alcohol is a central nervous system depressant and is most widely used, but it causes the most severe long-term damage to the liver.

Opioids

Also called as opiates, are either derived from the drug opium or chemicals designed to mimic it. Opioids work by interacting with neurotransmitters in the brain and blocking the signals that they are sending, serve as powerful pain killers, but can also cause feelings of intense pleasure Examples include heroin, fentanyl, oxycodone

Benzodiazepines

These function by interacting with the neurotransmitter gamma-aminobutyric acid-A (GABAA). Benzos are highly addictive, and can cause numerous medical and psychiatric problems when not used as intended, e.g. Ativan, Valium, Xanax.

Cannabinoids

These are chemically similar to tetrahydrocannabinol (THC), the active agent in marijuana. Examples include marijuana, Hashish

Barbiturates

They act on the central nervous system by slowing down its functioning, e.g. Amytal, Lumital, Phenobarbital

Pharmacological classification

Depressants

These agents are more commonly stated as “downers,” and create feelings of relaxation and tiredness. These are drugs that slow down the central nervous system and subsequently results in slowing down the messages between the brain and the body. This leads to diminishing concentration and reduced ability to respond. The word depressant might be misthought/ misunderstood as leading to a person becoming depressed, however it is not the case. These are very commonly abused because they may also create feelings of euphoria. Depressants are not only some of the most highly addictive drugs, but they are also some of the most highly dangerous chemicals likely to cause overdose. Examples of depressants include alcohol, opioids (opium or synthetic drugs drawn from opium like heroin).and barbiturates.

Stimulants

Also identified as “uppers,” the primary use of stimulants is to increase energy, concentration, and wakefulness. Contrary to depressants, stimulants are the drugs that speed up the central nervous system. They result in fast travel of messages between brain and body. They are also known as psychostimulants. They result in increased energy levels, heart rate and appetite and said to provide a “rush.” In the short term, stimulants are believed to increase productivity and performance, while producing an excited high of pleasure. In the long term, stimulants are incredibly addictive and have a very high potential for abuse. Examples of stimulants include nicotine, caffeine, Adderall, cocaine, methamphetamine.

Hallucinogens

Hallucinogens are sometimes referred as dissociatives. They change the way one sees the world, alter the user’s perception of reality, often resulting in auditory and visual hallucinations, a process known as “tripping.” Additionally, these drugs can change the way a person tastes, smells, hears and feels the different things and can also cause one to see or feel things which are not there at all. Although hallucinogens are generally less addictive than other drug classifications, their immediate impacts are generally more severe and dangerous. Examples of hallucinogens include Lysergic acid diethylamide (LSD), Psilocybin Mushrooms and phencyclidine (PCP), mescaline, ketamine etc.

Inhalants

Inhalants are a vast range of chemicals that are ingested primarily by breathing them in, or huffing. Most inhalants are commonly used materials that are in no way designed to be ingested by humans. While there is incredibly variety between inhalants, most produce feelings of a high. Inhalants are less studied than most other drugs. While they tend to be less addictive than many other substances, the use of inhalants is incredibly dangerous and causes many serioushealth effects. Examples of commonly abused inhalants include Paint thinner, Nail polish remover, Gasoline etc.

Legal classification

Schedule V
Schedule V drugs have the fewest regulations and lowest penalties of any federal drug classification. Schedule V drugs have a legitimate accepted medical purpose, have a lower potential for abuse than Schedule IV drugs, and have a lower potential for addiction than Schedule IV drugs. Examples include Lomotil, Motofen, Lyrica.

Schedule IV
Schedule IV drugs have regulations and penalties in between those of Schedule V and Schedule III drugs. Schedule IV drugs have a legitimate accepted medical purpose, have a low potential for abuse, and have a low potential for addiction. Examples include Ambien, Darvocet, Tramadol.

Schedule III
Schedule III drugs have more regulations and harsher penalties than Schedule IV drugs and fewer regulations and less severe penalties than Schedule II drugs. Schedule III drugs have a legitimate acceptable medical purpose, have a lower abuse potential than Schedule I and II drugs, and have a moderate or low potential for addiction. Examples of Schedule III drugs include Anabolic steroids, Ketamine, Vicodin.

Schedule II
Schedule II drugs have more regulations and harsher penalties than any drug classification other than Schedule I drugs. Schedule II drugs have a legitimate accepted medical use, a high potential for abuse, and a severe dependence risk. Examples of Schedule II drugs include Codeine, Methadone, Ritalin.
Schedule I
Schedule I drugs have the most regulations and harshest penalties of any drugs. Schedule I drugs have no legitimate accepted medical use and a high potential for abuse. Examples of Schedule I drugs include Ecstasy, Quaaludes, Gamma-hydroxybutyric acid (GHB), LSD

ANTIBIOTIC ABUSE

This refers to the misuse or overuse of antibiotics, with potentially serious effects on health. Antibiotics are sometimes used even when they are not needed. It is a contributing factor to the development of antibiotic resistance, including the creation of multidrug-resistant bacteria, informally called "super bugs". Relatively harmless bacteria (such as staphylococcus, enterococcus and acinetobacter) can develop resistance to multiple antibiotics and cause life-threatening infections. Examples include use of antibiotics in some viral infections such as cold, flu, in skin infections such as eczema. Use of antibiotics in animal husbandry and in agriculture as plant growth promoters should be avoided.

STEROID ABUSE

People abuse steroids for a variety of reasons, but most do it to increase body performance and appearance. Anabolic steroids are synthetic drugs that resemble the male sex hormone **testosterone**. in September 1988 in Seoul, Korea, the world of sports was transformed forever. Ben Johnson ran the 100-meter race in 9.79 seconds to win the Olympic gold medal and break the world record. He had become the world's fastest human—of all time. Then, just two days later, after traces of the **anabolic steroid** stanozolol were found in Johnson's urine sample, he was stripped of his record and his medal and left the games in disgrace. The aftershocks in the sports world are still being felt; perhaps more significantly, Johnson's scandal opened the eyes of the public to the problem of steroid abuse. Sports world is becoming extremely competitive, there may be peer pressure to use anabolic steroids in some sports because of a fear that normal training will not be enough to succeed. There is also the perception that other athletes are abusing steroids and gaining an unfair advantage. People who use steroids to enhance their appearance by increasing muscle and decreasing fat may suffer from muscle dysmorphia or abnormal perception of their own body. Anabolic steroids are used by athletes for performance enhancement. Anabolic drugs such as androstenedione, dehydrochlormethyltestosterone (turinabol), metandienone (Dianabol), danazol, methyltestosterone (android), nandrolone (durabolin), oxandrolone (oxandrin), oxymetholone (anadrol) and stanozolol (winstrol) are misused for muscle build up and increased physical strength. Many times widely used muscle building supplements are contaminated by unlabelled anabolic steroids. Sports supplements are widely available over-the-counter and online. Most users report a mild euphoria when they use steroids, and increased energy levels also are noted. Less desirable are reports of increased irritability and aggressiveness, sometimes leading to violent behavior. Mood swings and even psychotic reactions have been reported.

The following are some of the side effects of steroid misuse-

In men common physical effects are acne, balding, and reduced sexual desire atrophy of the testes and a related decline in sperm count and enlargement of the breasts. Women may experience pronounced masculinizing effects from steroid use, and many of these effects are irreversible. They include growth of facial and chest hair, baldness, deepening of the voice, breast shrinkage, clitoral enlargement, and menstrual irregularities. Risk of heart disease, damage to liver, premature bone fusion causing stunted growth, and brain cancer are other effects.

HABIT FORMING DRUGS

Prescription drug abuse

Opioids-Oxycodone, codeine, fentanyl, meperidine
CNS depressants- Benzodiazepines [alprazolam (Xanax) clonazepam (Klonopin), Diazepam (Valium)]
Stimulants- Amphetamine (Adderall), Methylphenidate (Ritalin)
Birth control pills-
Anabolic steroids- Metandienone (Dianabol), danazol, methyltestosterone (android), nandrolone (durabolin), oxandrolone (oxandrin),

Over the counter drug abuse

Over the counter (OTC) drugs are those which can be purchased from a medical or drug store without a prescription. OTC drugs are typically safe if used at recommended doses, however Like illegal and prescription drugs, they can also be abused. Although less potent than other substances, OTC drugs still pose a risk for developing an addiction.

Cough medicines- Dextromethorphan, or DXM
Cold medicines- Pseudoephedrine, diphenhydramine (Benadryl)
Motion sickness pills- Dimenhydrinate
Analgesics- Actaminophen (Tylenol)
Antacids- Milk of magnesia
Sleeping aids
Diet pills

METHODS OF TAKING DRUGS

The drugs could be taken by a person in one or more of the following manners-
1. Orally in the form of pills or powder.
2. Intravenously in the form of injections.

3. Inhaling
4. Snorting through nostrils.

EFFECTS OF DRUG ABUSE

Drugs alter the way nerve cells normally send, receive and process information. This could happen due to various factors such as i) drugs imitate the brain's natural chemical messenger, ii) drugs over stimulate the reward circuit of brain, iii) Drugs flood the brain with excess chemicals like dopamine, iv) They get bound to the receptors in the brain. The effects of drug abuse are determined by the type and quantity of drug taken. Duration for which the drug is consumed and individual characteristics of the person such as body size, present health status etc. also prominently affect the resultant effects of drug abuse. Illicit use of drugs may result in any one or more of the following: -

1. Health of the person using drugs get affected adversely over the long time use of drugs. The person starts feeling les energetic, may also stop feeling pain. At times the drugs affect the heath so badly that it may result in loss of life.
2. A person using drugs may indulge in sharing the needles with other addicted persons for use of drugs. This may lead to some serious infections causing various types of health complications which may prove fatal.
3. Drugs are very expensive and regular use of the drugs over a long time requires a lot of money. This may lead to some serious financial problems as there could be a loss of job or business because of the person being involved with drugs.
4. One may land himself in some legal complications as most of the drugs are illegal and use or possession of these drugs is a punishable offence.

VULNERABLE GROUPS

A key characteristic of drug abuse is that an individual continues to take drugs despite its negative impacts. Some reasons which could be contributing to a person continuously using the drugs are given below-

1. Genetic vulnerability

Several studies confirm that a person might inherit the habit of consuming drugs as there might have been someone in the family who was used to certain drugs.

2. Emotional disturbances

A person suffering from some emotional setback might go for a drug as a "quick fix" to get immediate satisfaction and escape from misery. It is said that alcohol makes one forget his worries. But overtime the brain of the individual adjusts to the continuous consumption of alcohol which results in to anxiety and irritability. That is the stage when a person starts drinking alcohol to feel normal instead of drinking to feel good.

3. Lack of alternative rewards

The lack of non-drug alternatives to escape miseries might be another reason for a person to keep using drugs. For example, a person living in poor neighbourhood and deprived of options to get pleasure or escape from maladies, has a great chance of indulging in drug use which eventually becomes abuse.

4. Impaired insight

Lack of awareness about the problems associated with use of drugs could be another reason for people to continue taking drugs.

5. Overvaluation of immediate reward

A person using drugs sees much more benefit in the immediate pleasure or relaxation the drug is producing as compared to the long term effects.

SOME WORLDWIDE FACTS AND FIGURES ABOUT DRUG ABUSE

World health organization (WHO) report 2019 reveals an estimated around 271 million people (which constitutes 5.5 % of world population) aged 15 to 64 years used illicit drugs at least once in a particular year 2017 and the number is increasing day by day. The harmful use of alcohol results in 3.3 million deaths each year. Approximately 31 million people have drug use disorders. Almost 11 million people inject drugs, of which 1.4 million are living with HIV, 5.6 million with hepatitis C, and 1 million with both HIV and hepatitis C. The Report also estimates the number of opioid users at 53 million, up 56 per cent from previous estimates. About 75 % of deaths associated with drug abuse are caused by the use of opioids, i. e. the opium related drugs like heroin.

Indian Scenario

In India about 2.6 crore people use opioids and more than 60 lakh people suffer from opioid related disorders. Approximately 1.80 crore people use inhalants and sedatives out of which about 4.6 lakh children and 18 lakh

adults have disorders related to these. About 8.50 lakh people inject drugs and 27% of them have admitted that they shared their needles with other drug users which puts them at risk of contracting diseases like HIV. About 14.60 % of the total population of India is known to use alcohol and about 6 Crore people suffer from harmful effects of alcohol. Indian states like Uttar Pradesh, Punjab, Delhi, Gujarat, Madhya Pradesh, Rajasthan, Maharashtra contribute significantly to these figures.

DRUG ADDICTION

What is drug addiction?

Let us first differentiate between the terms 'Drug abuse' and 'Drug addiction'. Many people use these terms mutually, however drug abuse starts first, and then subsequently leads to the disease called drug addiction. People who abuse certain drugs still have control, but addiction to drugs becomes a disease. One of the worst consequences of drug abuse is that it leads to drug dependence or drug addiction. The word addiction comes from Latin word addictus which means enslaved by or bound to. Drug addiction can be described as a disease caused by excessive intake of alcohol or drugs. Drug addiction is also known as substance use disorder. It affects a person's brain and behaviour in such a way that he is unable to control the use of any drug or medication, be it legal or illegal. We may also say that this is a disease resulting from changes in brain chemistry due to compulsive use of drugs. Different drugs act on the brain in different ways, but they all cause release of the neurotransmitter dopamine in the brain's reward area, resulting in the pleasurable sensation, common idiom for which is known as 'THE HIGH'. It is this sensation of 'HIGH' which forces a person to keep using drugs regularly. That is how the drugs become addictive for a person. Drug addiction may lead to –

1. Craving: A person gets so used to any particular type of drug that it becomes very difficult for him to live without it. At all the times he feels the desire to have that particular substance. We can co relate this with a simple example. If a person goes to the market and have some snack at say about 6 PM for 3 or 4 days in succession, he will automatically develop a craving for that particular snack and will be seen standing at that shop daily at the same time.

2. Loss of self-Control: When a person starts abusing the drugs he reaches a stage where he loses the self-control on amount and frequency to use the drugs.

3**. Compulsive disorder**: After losing self-control on use of drugs, it becomes a compulsion for him to use the same. He reaches a stage where he cannot think of living without the drugs.

4. Consequences: Most of the persons using drugs are aware of their adverse consequences on health, relations, finances etc, but when the use of drugs becomes a compulsion for them they knowingly or unknowingly ignore these consequences and continue using drugs.

RISK FACTORS FOR DRUG ADDICTION

Vulnerability of a person to get addicted to drugs differs from person to person. One's genes,mental health, family and social environment do play a considerable role. Risk factors that increase one's vulnerability include:

1. Family history of addiction
2. Abuse, neglect or other traumatic experience
3. Start of drugs at early age
4. Mental disorders such as depression and anxiety
5. Method of administration of drug. Smoking and injecting a drug increases the chances of addiction.

SYMPTOMS OF DRUG ADDICTION

The following conditions help find whether a person is addicted to drugs or not.

i) The person addicted to the drugs will feel to use the drug regularly, may be once or even more in a day.
ii) He needs for more quantity of drug as time passes in order to get the same HIGH he was getting initially with small doses and always try to maintain a regular supply of the drug even if find difficult to afford by hook or crook even stealing.
iii) He usually cuts back on social and recreational activities and find it difficult to meet social and professional obligations and responsibilities.
iv) The person finds it difficult to survive without the required dose of drug.
v) One start feeling lack of energy or motivation. He may also start gaining or losing weight beyond normally accepted limits.
vi) One may start losing interest in clothing, grooming or looks.

SIDE EFFECTS OF DRUG ADDICTION

Excessive use of drugs may result in to any or more of the following. A person addicted to drugs is likely to suffer Seizures, stroke, mental confusion, brain damage, Lung disease, problems with memory, attention and decision-making, which make daily living more difficult. Global effects of drugs on the body, such as breast development in men and increases in body temperature, can lead to other health problems.

1. Health of the person using drugs get affected adversely over the long time use of drugs. The person starts feeling les energetic, may also stop feeling pain. At times the drugs affect the heath so badly that it may result in loss of life.
2. A person using drugs may indulge in sharing the needles with other addicted persons for use of drugs. This may lead to some serious infections causing various types of health complications which may prove fatal.
3. Drugs are very expensive and regular use of the drugs over a long time requires a lot of money. This may lead to some serious financial problems as there could be a loss of job or business because of the person being involved with drugs.
4. One may land himself in some legal complications as most of the drugs are illegal and use or possession of these drugs is a punishable offence.

WITHDRAWAL FROM DRUGS

Withdrawal can be described as the physical and mental effects a person experiences when he stops or reduces the intake of drugs abruptly. Withdrawal is also known as detoxification. It is a stage when one reduces or stops the use of drugs abruptly. It results in to abnormal physical and psychological behaviour in a person when he discontinues the use of drugs. The person going through withdrawal of drugs is likely to feel sleeplessness, fever, chills and sweat, running nose, nausea, vomiting, muscle aches and cramps. When a person has used a drug for a long time, he is likely to experience more severe withdrawal symptoms as the body will take a long time in coming to the pre drug use state. In some cases, these symptoms are so severe that they last for a long time and become more or less permanent. To avoid the above conditions, it is always better to withdraw from the drugs under medical supervision.

Withdrawal symptoms

Withdrawal symptoms are often opposite to the effects of the substance. For example, alcohol is a depressant, so if one suddenly stops consuming

alcohol, he might experience symptoms of overstimulation such as anxiety or restlessness. When a person is using drugs regularly his brain acts like a spring which has some weight put on it. When that person stops using the drugs, it takes that weight off the spring and the brain rebounds by producing a lot of adrenaline which causes withdrawal symptoms. When a person has used a drug for a long time, he is likely to experience more severe withdrawal symptoms as the body will take a long time in coming to the pre drug use state. In some cases, these symptoms are so severe that they last for a long time and become more or less permanent. Withdrawal symptoms vary depending upon the type of drug one was using and the duration for which it was used.

Physical symptoms- The physical symptoms of withdrawal might last only a few days or a week, Some symptoms commonly associated with withdrawal include irritability, changes in appetite, changes in moods, fatigue, nausea, restlessness, running nose, shakiness or trembling, sleeping difficulties, vomiting. In some cases there can be more severe symptoms such as delirium or hallucination.
Psychological symptoms- The psychological can last much longer and include depression or dysphoria.

Withdrawal symptoms and their expected duration with regard to some specific substances is as follows:

Alcohol: Alcohol withdrawal symptoms are not seen in everyone who stops drinking, but most people who quit suddenly after drinking enough alcohol for any length of time can experience a wide range of symptoms. Many times, those symptoms will trigger a relapse.
Heroin: Those who have become addicted to heroin experience some particularly intense withdrawal symptoms, but even the worst of those symptoms will subside in five to seven days. However, for some it can last for weeks or even months.
Marijuana: Compared to alcohol and other drugs, the withdrawal symptoms for some marijuana users are on the mild side. But, some of those symptoms are so unpleasant that they decide to go back to using the drug.
Nicotine: Not everyone experiences all of the same symptoms of nicotine withdrawal. As many who smoke know, symptom of nicotine withdrawal can make it difficult to give up cigarettes.
OxyContin (oxycodone): The severity of withdrawal symptoms related to OxyContin and other prescription opioids depends upon the amount and duration of drug use. If one has taken painkiller only as directed, he may not experience any withdrawal symptoms at all, or very mild ones.

TREATMENT OF WITHDRAWAL EFFECTS

To avoid the withdrawal effects, it is always better to withdraw from the drugs under medical supervision. Treatment for withdrawal includes support, care, and medications that can ease symptoms and prevent possible complications. With some substances, people are able to stop their use abruptly and manage their withdrawal symptoms on their own. For example, a person may be able to quit caffeine without assistance and cope with the unpleasant symptoms on his own. But abruptly quitting substances such as benzodiazepines or alcohol can be potentially dangerous. So one must consult the doctor so that he can suggest a suitable detox plan. Medically-assisted withdrawal can ensure that one is safe and it helps to minimize unpleasant withdrawal symptoms.

COPING UP WITH WITHDRAWAL

In addition to seeking medical support, the following points should to be considered by a person to go through the withdrawal process smoothly.

Ask for help. Whether one is handling withdrawal on his own or under the supervision of a doctor, it is important to have social support. One should tell a trusted friend or family member so that they can check-in and support during the process.

Take proper diet. One must focus on eating nutritious, well-balanced meals. Eating fried, fatty, or sugary foods may make one feel worse.

Exercise. One should try to get some physical activity each day. Stretching, walking, swimming, or other activities may help boost the mood.

Drink plenty of water. It is important to stay hydrated while going through withdrawal, especially if there are flu-like symptoms such as nausea and vomiting.

Relieve symptoms with over the counter (OTC) medications. Use appropriate OTC medications at the recommended dosages in case of symptoms such as headache, upset stomach, or diarrhoea.

Sleep. While withdrawal can sometimes lead to sleeping difficulties, one should try to get an adequate amount of rest.

Yoga and meditation. Stress management activities such as yoga and meditation are also helpful in coping with withdrawal experience.

INITIATIVES OF GOVT IN INDIA

1. Under the National Drugs and Psychotropic Substances Act (NDPS Act) the Government can take action against a person for cultivation, production, possession, sale, purchase, trade, import, export, use and consumption of narcotic drugs and psychotropic substances except for medical and scientific purposes in accordance with the law.

2. NDPS Act also supports treatment of people using drugs.
3. A national fund for control of drug abuse was established in India. This fund can receive contributions from central Government, individuals and proceeds from sale of property belonging to drug traffickers.
4. NGO's and Govt departments can apply to this fund for getting provisions for activities related to drug control which may include treatment and spreading awareness against drugs.
5. NDPS Act also provides for setting up of drug de addiction centres. These centres are set up by Central and state Governments and also by voluntary organisations.

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CHAPTER 2

PREVENTION AND MANAGEMENT OF DRUG ABUSE

Prevention and Management of drug abuse or drug addiction- Drug addiction treatments are commonly used among those patients who are being arrested, locked up, unable to withdraw from drugs for several months. However, this is not always the case. Indeed, addiction is a brain-specific problem that is driven by biological and environmental factors. Recent research on drug abuse has shown that addiction has many different origins, including inherited traits (genetically and epigenetically), environmental/social pressure, personal habits, and other indeterminate causes. Thus, to treat addiction one must face a myriad of causes and a lifestyle change involving both medical (pharmacological) and behavioral (psychological) treatments.

Treatments for drug addiction and withdrawal symptoms can take several forms and degrees of effectiveness. Some users require along withdrawal period, and others withdraw quickly after stopping drug use, while some do not respond to any form of treatment. Treatment does not require total abstinence (stoppage) but can be considered successful with a reduction in drug use. Long-term drug use provokes changes in the brain systems (discussed above), thereby hindering abstinence, both psychologically and physiologically.

It is important to underscore that there is no single, highly effective treatment that can be universally applied. Different treatments have to be used in sequence or simultaneously to achieve success, and sometimes it takes longer to discover the best treatment approach for drug addiction.

Management of drug addiction includes treatment as well as prevention. Scientific evidence indicates that the development of drug use disorders and dependence is a result of a complex multi-factorial interaction between repeated exposure to drugs and biological and environmental factors. Effective treatment typically incorporates many components — pharmacotherapy (Drug therapy), behavioral therapy, and anti-stress or mindfulness-based support — each directed towards a particular aspect of the disorder and matching an individual's particular problems and needs.

A) Drug Treatment- Following drug treatment has been suggested to treat drug addiction of different types-

1. For alcohol addiction- The most common medications for alcohol dependence are disulfiram, acamprosate, and naltrexone

2. For Tobacco addiction- Nicotine patches, gum, lozenges, and nasal spray and the medications varenicline and bupropion are used.
3. Opioid addiction- Methadone, buprenorphine, and naltrexone are used.

B) Psychological treatment or Behavioral 19therapy)

a)Motivational Enhancement Therapy- Motivational Enhancement Therapy (MET) is based on the trans-theoretical model of behavior change which postulates that change in behavior takes place in different stages. MET uses motivational interviewing to enhance treatment readiness and move the patient through the stages of change (pre-contemplation, contemplation, determination, and action) for evoking change in substance use behaviors. MET is characterized by an empathic approach in which the therapist helps to motivate the patient by asking about the pros and cons of specific behaviors, exploring the patient's goals and associated ambivalence about reaching those goals, and listening reflectively to the patient's response. The key elements of MET which are believed to be the active ingredient of the intervention are summarized as

i.Feedback of personal risk or impairment ii. Emphasis on personal responsibility for change iii. Clear advice to change iv. A menu of alternative change options v. Therapist empathy vi.Facilitation of client optimism

b) Behavioral Therapies- It is based on the principles of learning; the target behavior of habitual excessive substance use is altered through systematic environmental manipulations that vary widely depending on the specific substance use behavior. Contingency contracting is a subtype of contingency management based on the use of predetermined positive or negative consequences to reward abstinence or punish, and thus deter drug-related behavior. Negative consequences of substance use may include notification of courts, employers, or family members. Cue exposure treatment involves exposing a patient to cues that induce craving while preventing actual substance use and, therefore, the experience of substance-related reinforcement. Cue exposure can also be paired with relaxation techniques and drug-refusal training to facilitate the extinction of classically conditioned craving. Aversion therapy involves coupling substance use with an unpleasant experience such as mild electric shock, pharmacologically induced vomiting, or exaggerated effects of the substance. This treatment seeks to eliminate substance use behaviors by pairing them with punishment.

c) Cognitive-Behavioral Therapies (CBT) -Social skills training, an element of CBT, recognizes that alcohol and drug dependence commonly result in the interruption of normal developmental acquisition of social skills as well as the deterioration of previously learned social skills because of the

interference of drug-seeking and drug-using behaviors. Social skills training targets an individual's capacity for effective and meaningful communication including nonverbal communication, listening, being able to think from others' perspectives, adapting to different circumstances, maintaining relationships, and being assertive. Relapse prevention is a treatment approach in which CBT techniques are used to help patients develop greater self control to avoid relapse. Specific relapse prevention strategies include discussing the patient's ambivalence about the substance use disorder, identifying emotional and environmental triggers of craving and substance use, developing and reviewing specific coping strategies to deal with internal or external stressors, exploring the decision chain leading to the reinitiating of substance use, and developing effective techniques for early intervention.

d) Group Therapy- Group therapy is viewed as an integral and valuable part of the treatment regimen for many patients with a substance use disorder. Many different types of therapies have been used in a group format with this population, including CBT, IPT, and behavioral marital, modified psychodynamic, interactive, rational emotive, Gestalt, and psychodrama therapies. Given the social stigma attached to substance use disorders, the presence of other group members who acknowledge having a similar problem can provide comfort. Besides, other group members who are further along in their recovery can act as models, and provide hope and encouragement.

e) Family Therapies- Dysfunctional families, characterized by impaired communication and an inability of family members to set appropriate limits or maintain standards of behavior, are associated with poor short- and long-term treatment outcomes for patients with substance use disorders. The goals of family therapy include obtaining information about the patient and his factors that contribute to substance abuse. These include the patient's attitudes toward substance use, treatment adherence, social and vocational adjustment, level of contact with substance-using peers, and degree of abstinence. Family support for abstinence, maintaining marital and family relationships are encouraged. Even the brief involvement of family members in the treatment program can enhance treatment engagement and retention. Controlled studies have shown positive outcomes involving non-alcohol-abusing family members in the treatment of an alcohol-abusing individual.

f) Alternative Treatments -Stress is a significant contributor to drug abuse and relapse. It is known to increase drug use in general and alcohol and cigarettes in particular. However, recent research suggests that mindfulness-based cognitive therapy, physical exercise, and/or an alternative pharmacological intervention are promising in drug addiction treatments. Mindfulness reduces stress and has a potential impact on drug use and relapse. The technique is based on the user's awareness and acceptance of their experience and interruption of the craving/using/relapse cycle. It teaches how to process situations that may lead to relapse, inducing users to

monitor their internal state, and react using mindful awareness, thereby making positive choices.

DRUG ADDICTION CENTRE

History and genesis- The Constitution of India, under Article 47, enjoins that the state shall endeavor to bring about prohibition of the consumption of intoxicating drinks and drugs, which are injurious to health. The activities to reduce the drug use related problems in the country could broadly be divided into two categories- supply reduction and demand reduction. The supply reduction activities which aim at reducing the availability of illicit drugs within the country come under the Ministry of Home Affairs, with the Department of Revenue as the nodal agency. The demand reduction activities focus upon awareness building, treatment, and rehabilitation of drug-using patients. These activities are run by the Ministry of Social Justice and Empowerment (MSJE) as the nodal Ministry and to some extent by the Ministry of Health and Family Welfare through its Drug De-Addiction Program (DDAP). The Ministry of Health & Family Welfare is mainly involved in providing treatment services to the addicts whereas the Ministry of Social Justice & Empowerment deals with other aspects of the problem like awareness creation, counseling, and rehabilitation. Drug De-addiction Programme (DDAP) was started in 1985-86 and came into force in 1988 with the objectives to provide affordable, easily accessible, and evidence-based treatment for all substance use disorders through the government health care facilities and to build the capacities of health care staff in recognition and management of substance use disorders. The program was modified as a scheme in 1994 which was later revised again in 1999. The scope of the scheme was enlarged to include assistance to State Governments/Union Territories for developing Deaddiction Centers in identified medical colleges/district-level hospitals. The Ministry of Health & Family Welfare operates a Drug De-Addiction Programme (DDAP) by providing financial grants for augmenting post abuse treatment facilities in selected Central Government Hospitals/ Institutions and the Government Hospitals/ Institutions. Under this program, a National Nodal Centre, the "National Drug Dependence Treatment Centre (NDDTC), Ghaziabad (U.P.)", has been established under the All India Institute of Medical Sciences (AIIMS), New Delhi. The other DDTCs under this program include PGIMER, Chandigarh, and NIMHANS, Bangalore. The purpose of these centers is not only to provide de-addiction services and rehabilitation services to the patients but also to conduct research and provide training to medical doctors in the area of drug de-addiction. The Drug Treatment Clinics (DTC) scheme is another strategy for enhancing the provision of treatment services coordinated nationally by the NDDTC, AIIMS and different DTCs are functional in different states in the country

SERVICES AT DRUG DE-ADDICTION CENTER

i) Registration - At a minimum level, all patients attending de-addiction treatment services should be registered in a dedicated register and should receive a unique registration number. This service is linked to record maintenance and thus patients unique registration number should be reflected in all the records of the patient. While most hospitals (of which the deaddiction centers are a part) are expected to have a central registration system, the deaddiction registration number should be separate from the hospital registration as this would be important for monitoring and evaluation purposes.

ii) Outpatient Treatment – Given the dearth of the specialist medical human resources in the country, the de-addiction services would have to be provided by doctors who may be either General Duty Medical Officers (GDMOs) or medical specialists/physicians. While these doctors may encounter many patients with substance use disorders in their routine –general clinics, such patients should be referred to / encouraged to attend, the exclusive and dedicated outpatient clinic for substance use disorders. Thus, all hospitals with Government De- Addiction centers should offer an exclusive outpatient De-addiction clinic. The expected patient load would determine the frequency of this De-addiction outpatient clinic; it may vary from daily (i.e. on all working days) to once/twice/thrice a week. The following services should be available in a De-addiction center at the outpatient level:

a) Assessment: All patients should undergo clinical assessment (i.e. history taking and examination) by a trained and competent doctor. The assessment should be geared at making a clinical diagnosis (as per the ICD-10 or DSM-IV guidelines) as well as formulating a treatment/intervention plan. For the purpose, adequate infrastructure should be available ensuring comfort and privacy for the patients.

b) Counseling / psychosocial interventions / psycho-education: All patients (and their attendants, if available and only if the patients agree to involve them) assessed by the trained doctor, should receive Counselling / psychosocial interventions / psycho-education, as per the clinical needs. For this purpose, it would be necessary to involve a trained medical social worker/counselor/psychologist.

c) Treatment prescription: Every patient should receive a prescription of the treatment advised to him. If the procedures for dispensing involves a dispensing slip, that may also be provided to the patient. The outpatient services should have provisions for both – the new patients as well as for the old patients on follow-up.

iii) Inpatient Treatment - Patients who require in-patient treatment should be admitted in a dedicated ward which is exclusively meant for this purpose. Thus, each hospital with a government de-addiction center should have an exclusive, 10-bedded de-addiction ward. While the duration of the in-patient treatment may vary as per the individual needs of the patients, all efforts must be made to provide the in-patient treatment for an adequate length of time. During the in-patient stay, the following services should be made available to the patient:

- Assessment by the doctor(s): At least once per day during the morning rounds.
- Availability of nursing care: round the clock
- Availability of emergency care (on-call doctor): round the clock
- Psychosocial interventions
- Medicines-For the treatment of withdrawal symptoms and for the management of associated conditions/symptoms
- Food
- Facility to meet visitors during the specified visiting hours
- Access to facilities for recreation: newspapers, television (if available), indoor games

Since most patients admitted in the de-addiction ward would be suffering from withdrawal symptoms, all efforts must be made to make their hospital stay comfortable. The in-patient treatment period should be used to formulate the plans for long-term treatment and rehabilitation and the same must be discussed with the patient. All admitted patients should be provided with a discharge summary with a detailed plan for further treatment from the OPD.

iv) Dispensing of Medicines- Pharmacotherapy plays a central role in the treatment of substance use disorders. All the patients seeking treatment from de-addiction centers should have access to the following medications – free of cost – from the dispensary. Besides, the center should strive to also make available medications listed as ―Other medications‖ , though they have not been put on the essential list. If the general hospital, of which the deaddiction center is a part, has a dispensary, the same may be used for the de-addiction center as well. Otherwise, for dispensing of medications from the De-addiction OPD, a system should be put in place, which allows for monitoring and auditing the dispensing procedure. It must be remembered that some of the medications used for treatment possess abuse liability and the risk of diversion. Only authorized persons (such as a nurse/pharmacist) must be allowed to handle/dispense medicines. In a single visit, dispensing of take-home medicines for a period longer than two weeks should be

avoided. SOPs for ensuring regular procurement, storage, and dispensing of medications must be in place.

v) Emergency Services- In those de-addiction centers which are a part of the general hospital, emergency de-addiction services may be provided by the emergency department of the hospital. While most such emergency set-ups have a provision of doctors/nurses and necessary supplies, these should be geared towards providing emergency services related to substance use disorders. The emergency set-ups should be geared to handle emergencies commonly encountered in the area of substance use disorders viz. acute intoxication/overdose, withdrawal syndromes, adverse drug reactions, etc.

vi) Laboratory services- All the de-addiction centers should have access to basic laboratory services. These services don't need to be established exclusively for de-addiction patients but these services may be incorporated with laboratory services of the general hospital.
Specifically, the facilities for the following investigations should be made available:

- Routine Hemogram i.e. Hb, TLC, DLC, ESR, Platelets
- Liver function tests i.e. Serum Bilirubin, AST, ALT, Serum Alkaline Phosphatase, γ-GT
- Routine biochemistry: Blood Sugar, Blood Urea, Serum Creatinine, Routine Urine chemistry
- HIV screening: through linkages with associated ICTC
- Chest X-ray
- ECG
- Additional services (if possible): Ultrasound abdomen, Urine screening for drugs

vii) Psychosocial interventions- All centers should be equipped with facilities to provide psychosocial interventions at both the levels of care: Outpatient and In-patient and in both the settings: in group settings and individual settings. Family members must also be involved in psychosocial interventions as much as possible. While the specialized psychotherapies may be out-of-scope for most of the centers, trained manpower and other facilities must be available for the following psychosocial services:

- Basic psycho-education about the nature of the illness, important of treatment adherence
- Motivation Enhancement
- Reduction of high-risk behavior
- Brief Interventions
- Relapse Prevention
- Counseling for occupational rehabilitation

viii) Referral / Consultation / Linkages- While a comprehensive treatment program should address the multiple needs of the patient, no single de-addiction center alone can provide all the services a patient requires. Consequently, the centers must establish and maintain referral and consultation linkages with other facilities and services. The important ones are:
General and specialized medical services- for associated co-morbidities. Since the deaddiction centers are a part of the general hospital, it is expected that many of the specialist medical services would be available. For any associated symptoms / medical condition, appropriate referral or consultation from the concerned specialty must be sought. This is especially important for conditions like HIV/AIDS (linkages with ART center), Tuberculosis (linkage with DOTS center), and psychiatric conditions (linkage with the nearest psychiatric facility or District Mental Health Programme – DMHP).

ix) Paramedical staff (ASHA, Health worker, ANMs, Anganwadi workers)- These are the paramedical staff that is usually expected to be in close touch with the community. If these staff members come across out-of-treatment drug users in the community, they can motivate the drug users to seek treatment at the Government De-addiction center and provide a referral.
x) NGOs in the locality- It is well known that most substance users are unable to access treatment services for a variety of reasons. If there are NGOs in the locality working with substance users the center should strive for establishing linkages with them so that NGOs could identify the out-of-treatment drug users in the community and encourage them to seek help at the Government De-addiction center. Similarly, after the early phase of treatment is over from the Government De-addiction center, NGOs could help the patient in social/occupational rehabilitation.
xi) Record Maintenance- Maintenance of clinical records is a very important aspect of the functioning of the center. It is not only necessary for adequate clinical care of a patient at the individual level, but it is also necessary for monitoring and evaluation purposes at the program level. The following records should be maintained by a center:

xii) Individual patients file- This should be traceable by a unique registration number. A system should so develop that, once the patient is admitted and discharged, his/her inpatient treatment record is also incorporated in the out-patient file. Thus, upon pickingup a patient's file, one should be able to assess the entire treatment history and progress of the patient in chronological order. At each instance of patients follow-up in OPD, the file should be retrieved, presented to the clinical staff (doctor/counselor), in which necessary clinical notes should be entered. All the records related to the patient (Investigation reports etc.) should be attached to the file.

xiii) Drug Abuse Monitoring System (DAMS) proforma- At every instance of first registration in the de-addiction clinic, the doctor/counselor should fill up this proforma. Periodically, these proformas would be collected by a designated central/regional center and would be used to analyses the profile of patients seeking treatment.
xiv) Medication dispensing records- These should be maintained very stringently since some of the medications used for treatment possess abuse liability and risk of diversion. A system should be developed which would make each unit of medicine entering into the stores accounted for. After dispensing, the patients should be asked to sign the receipt of the medications.

Various Stages of treatment at de-addiction Center

1. Admission phase or patient recruitment- During these phase patients are encouraged to take up treatment and boost their confidence towards the benefits of treatment. They are generally convinced by proper counseling and removal of their doubt or myths about treatment. Counseling is generally based on their past drug history, health record/ medical record, and social behavior. A declaration bond is signed among patient, family, or legal representative and Centre.

2. Treatment phases

i. Detoxification stage- based on the condition of the patients, different drugs are used to prevent withdrawal symptoms. This phase aims to prevent the use of habit-forming drugs with some other which does not have habit-forming. This takes generally one to two weeks' duration. This also includes other supporting therapy such as motivational or behavioral counseling.

ii. Deaddiction stage- in this phase, addicted patients are given other supporting interventions such as recreation, educative session, family role, and reestablishment of link with family, society, and community and development of confidence to perform their day to day activity. The intervention of this phase different from patient to patient depending upon the severity of patients. This phase generally takes 2 to 3 weeks.

iii. Follow up stage- Once patients are discharged from the center, it is frequently being called to follow-up so that their problem can be solved when they start interacting with family, society, and community. So that they can be integrated easily and avoid de-addiction.

Prevention of drug addiction- The best way to prevent an addiction to a drug is not to take the drug at all without medical prescription and medical supervision or consultation. Follow he instructions provided by your doctor/ physician who prescribes a drug with the potential for addiction and its use care when taking the drug. Generally, a physician prescribes these medications at safe doses and amounts and monitor their use so that you would not be addicted to the drugs. The person must consult with a physician if a person takes more than the prescribed dose of a medication.

Preventing drug misuse in children and teenagers- the following steps may be helpful to prevent drug misuse in children and teenagers:

a) Communicate. Talk to children about the risks of drug use and misuse.
b) Listen. Try to be a good listener when children talk about peer pressure and are supportive of their efforts to resist it.
c) Set a good example. Don't misuse alcohol or addictive drugs. Children of parents who misuse drugs are at greater risk of drug addiction.
d) Strengthen the bond. Work on your relationship with your children. A strong, stable bond between you and your child will reduce your child's risk of using or misusing drugs.

How to prevent the relapse- Once a person has been addicted to a drug, he or she is at high risk of falling back into a pattern of addiction. If you do start using the drug, you'll likely lose control over its use again — even if you've had treatment and you haven't used the drug for some time.

a) Stick with your treatment plan- Monitor your cravings. It may seem like you've recovered and you don't need to keep taking steps to stay drug-free. But your chances of staying drug-free will be much higher if you continue seeing your therapist or counselor, going to support group meetings, and taking prescribed medication.
b) Avoid high-risk situations- Don't go back to the neighborhood where you used to get your drugs. And stay away from your old drug crowd.
c) Get help immediately if you use the drug again- If you start using the drug again, talk to your doctor, your mental health professional, or someone else who can help you right away.

Challenges for drug deaddiction center

Although the importance of the deaddiction center cannot be ignored. But it is still faced several challenges in current times. This includes-

i. No regular funding (except for centers in the North-East where the funding is available, though inadequate).

ii. The low priority given to de-addiction services by the state health departments/district/civil hospital authorities
iii. No dedicated staff available
iv. Lack of support staff – Nurse, social worker, counselor.
v. Poor patient load and treatment non-seeking
vi. Irregular supply of medicines. No medicines are available for long term use.
vii. Lack of community-based activities and linkage with NGOs
viii. Inadequate record-keeping or data management 9. No services available for women and adolescent substance users

SUPPORT OF FAMILY, MEDIA AND SCHOOL EDUCATION

1) Family- Support of family in the prevention of drug abuse or drug addiction- Each family member is uniquely affected by the individual having drug abuse but not limited to having unmet developmental needs, impaired attachment, economic hardship, legal problems, emotional distress, and sometimes violence being perpetrated against him or her. The family remains the primary source of attachment, nurturing, and socialization for humans in our current society. The family plays a key part in both preventing and intervening with substance use and misuse, both through inducing risk, and/or encouraging and promoting protection and resilience. Family members can counsel, educate, boost moral support including psychological need when a member detected, suspected, or identified with drug abuse or addiction problem:

The social work profession has well recognized the importance of assessing the individual in the context of his or her family environment. Social work education and training emphasize the significant impact the environment has on the individual and vice versa. Social work education is an effective way to help the family and the individual. The utilization of family based approaches has demonstrated superiority over individual or group-based treatments. Treating the individual without family involvement may limit the effectiveness of treatment for two main reasons
a) It ignores the devastating impact of drug abuse on the family system leaving family members untreated,
b) It does not recognize the family as a potential system of support for de-addiction.
Two theories namely attachment theory and systems theory are important to understand how drug abuse impacts the family-

Attachment theory- Relationships serve as the communication conduits that connect family members. Attachment theory provides a way of understanding the development and quality of relationships between family members.

Attachment theory postulated that at the time of an infant's birth, the primary relationship, usually with the mother but not always, serves as the template for all subsequent relationships throughout the life cycle. This relationship forms a subsystem within the larger family system. It is through this relationship, at a pre-language level, that infants learn to communicate and relate to their environment. They do this through crying, cooing, rooting, and clinging. How the primary caretaker responds to these cues will establish the quality of the attachment. Generally, if the child experiences the primary caretaker as responsive and nurturing, a secure attachment will form. If the child experiences the primary caretaker as unresponsive or inconsistently responsive, an insecure attachment may form that can result in a variety of problems including anxiety, depression, and failure to thrive.

The study's patterns of families with drug abuse or drug addiction problems revealed that significantly influence child development and the likelihood that a child will struggle with emotional, behavioral, or substance use problems. The negative impacts of drug addiction or drug abuse on the family include loss of attachment, rituals, roles, routines, communication, social life, and finances, etc. Family member (s) having drug abuse are well characterized by the frequent incidence of conflict, violence or abuse, emotional chaos, role reversal, and fear, etc.

Family Systems Theory- Family systems theory grew out of the biologically based general systems theory. General systems theory focuses on how the parts of a system interact with one another. In general systems theory, an individual cell is one example of a system, and in family systems theory the family is essentially its system. Key concepts in both theories are feedback, homeostasis, and boundaries that are defined and operationalized in this section.

All the family therapy models share the basic principle of family systems theory that is that the individual cannot be fully understood or successfully treated without first understanding how that individual functions in his or her family system. Individuals who present in our clinical settings can be seen as "symptomatic," and their pathology can be viewed as an attempt to adapt to their family system to maintain homeostasis.

Individuals with drug abuse problems cannot be understood and treated effectively without considering the impact on the whole family. Researchers have confirmed the reciprocal relationship between the disease of addiction and the environment. All persons influence their social environment and in turn, are influenced by it. The family system must be taken into consideration for a better understanding of the disease development and

maintenance as well as be included in the efforts necessary for successful ongoing treatment. The earlier the intervention, the better the outcomes for all family members.

Family Involvement and support of parents, guardians, and other caregivers are critical in preventing youth substance abuse. Conversations are one of the most effective tools adults can use to connect with youth. The youth of parents who talk to their kids about the dangers of substances early and often are 50 percent less likely to use drugs than those who do not receive these critical messages at home. A child may be more or less likely to try drugs due to certain circumstances. Below are risk factors that may increase vulnerability to using drugs and protective factors that reduce those risks.

Risk Factors	Protective Factors
Lack of parental supervision	Parental support and strong family bonds
Poor attachment with caregivers	Presence of mentors or adult role models
Academic problems	School and community engagement
Undiagnosed mental health problems	Coping and problem-solving skills
Substance-using parents, siblings or friends	Healthy peer groups
Peer rejection	High self-esteem
Child abuse/neglect	Stable home environment

Families can act as powerful protective forces in the healthy development of children and adolescents. Strong attachments between parents and children, supportive parenting and supervision, monitoring, and effective discipline have all been found to be linked to less engagement in problematic and risky behaviors during adolescent years. In particular, parents are a powerful protective factor for children and adolescents about substance abuse.

2. Media - The media play a very important role concerning the prevention of drug abuse or drug de-addiction program such as alcohol programs for young people. Media can broadly create both positive and negative impacts on society, particularly young generations. Positive imager impact of media

may include music, television, radio, and print materials influence the choices and behaviors of drug addicts. These modes of media influence significantly on both patients who are undergoing treatment in drug deaddiction center or those who are making their efforts to prevent the problem of drug abuse. Therefore, it is important to develop collaborative relationships with the media to achieve positive messages and outcomes for young people. On the other hand, the media can also create negative images of young people and influence how the rest of society views them. There are growing concerns about the influence of the media on young people, and, at times, problems such as drug and alcohol abuse, sexual promiscuity, and depression have been blamed on the media.

The media are also said to have a significant role in shaping youth lifestyles and cultures and marketing messages can play a key role that affects self-concept, body image, sexuality, and drug and alcohol consumption. Services need to educate young people in that regard, as well as work with the media to try to reduce the impact of such marketing. The media can also promote positive images of young people among the public. The media can be a useful tool to help change the negative attitudes that may exist towards young people, as well as an aid in trying to access funding and sponsorships. Relationships with the media are important to communicate the strengths of young people and to send accurate messages to all sections of society, including politicians. That will help to reshape myths and highlight the positive aspects of the participation of young people in the life of an inclusive and multicultural community. Many communities have their local media, such as newsletters and local community radio stations. Also, there are examples of educational street theatre or plays themed on drug and alcohol use. The involvement of the local community in scriptwriting, staging, and follow-up activities have been an effective strategy. Such activities can also be a valuable way of breaking down barriers in the community and educating large numbers of people on drug-related harm.

Various Modes of Media and their potential uses to prevent drug abuse

i. **Music-** Audio or video both types of music are powerful tools that can have an impact on large masses of patients having drug abuse problems. Different types of music and video help them to distract their attention towards drug craving. Their full involvement not only distracts their attention but also changes behavior and mood. Withdrawal symptoms are generally associated with the various neuropsychiatric problem among patients. Music therapy has well proven to deal with such problems during the prevention and management of drug addiction.

ii. **Television-** Television is one of the modes of mass communication that reaches and influences people significantly. Television has created both positive and negative images among the young generation. It is the fastest-growing medium of media. Now a day, every home and every person of all age groups watch television either news, movie, sports, etc.

iii. **Radio-** Radio has still its influence and impact among the rural area where people areliving in remote areas or below the poverty line. Several programs on FM are specially designed and telecast to deal with such problems among youth.

iv. **Print materials**- Print media includes newspapers, magazine, journal which frequently touches and deliberate on the issue of drug abuses. Therefore, print is also a powerful tool to create awareness in their native language among youth or population-dense with abusive properties.

v. **Film and Documentaries**- Film and documentary have more impact than television or radio or print media. Film and documentary involve stories, emotions, desires, and reflect the life of youth or social issues related to drug abuse. These films and documentaries are not only entertaining the people but also send their message to the community to prevent drug abuse. Besides, these types of media would help to rectify the social problem and their effective prevention.

vi. **Posters-** Poster has its impact when put in common areas or place where the majority of people see daily and their image and message is read by everyone. Daily watching or reading the poster has a positive impact on the brain of the person affected that drug abuse is a social problem and evil. Most of the time, posters are frequently used by NGOs and hospitals, government agencies to create wide circulation among people. These posters are put either in hospitals, in front of the cinema, market, main market, government website, buildings, notice board, etc.

ii. **Campaigns**- Campaign is another mode wherein a group of people sends their message in the form of drama, skid, debates competition highlight specific issues such as alcohol, cigarette smoking, drugs, etc. The campaign always has created a positive image among people. However, campaigns are less powerful in comparison to television or film, or documentaries. Campaigns are generally used as an educational activity to educate common people.

vii. **Folk media**- it is a powerful tool in rural or tribal areas where people understand their native language. Therefore, folk music is an effective medium to prevent drug abuse among rural and tribal areas.

3. School education- Child during school education is generally passing through different developmental stages such as elementary level, early (primary level), middle (secondary level), and late adolescence stage (senior secondary level). The primary developmental challenge in elementary school is to translate the self-regulation learned at home to the school environment and master academic and social situations. Middle childhood is marked by great advances in learning and understanding. During this developmental stage, the cognitive abilities of children increase rapidly. Children develop a clear understanding of right and wrong. Values and standards are primarily influenced by parents and teachers, as they are the primary source of reinforcement. In elementary school, drug abuse is rare, and expectancies about the effects of substances that are predominantly negative. Therefore, it is expected that substance use prevention in this developmental stage should not necessarily address the risks of substance use. Instead, it is expected that elementary school students will benefit most from programs teaching them basic skills helping them master their primary developmental challenges.

Early adolescents experience several biological, cognitive, and psychosocial changes that lead them to reappraise themselves and those around them. They have to adapt to their changing body, as they start to develop secondary sexual characteristics. At the same time, cognitive capacities develop dramatically. Early adolescents begin to develop the capacity to think abstractly, which allows them to think hypothetically and to evaluate multiple outcomes. Furthermore, the development of metacognitive abilities enables children toreflect on their thoughts and behaviors. They become increasingly concerned with peerrelationships. Early adolescents believe that others are always watching and evaluating them. At the same time, early adolescents believe that they are unique and invulnerable. This phenomenon is known as 'personal fable. These feelings help the early adolescent to perceive the self separately from family-ties, although parents remain an important source of reinforcement in this developmental stage. Besides, early adolescence is accompanied by profound environmental changes, as children make the transition to middle or secondary school. The new school environment is characterized by multiple classes and teachers, less individualized instructions, lower levels of teacher-student interactions, more stringent grading, and comparative performance evaluations. Early adolescents spend significantly more time with peers and, as a result of frequent class changes, are exposed to larger numbers of peers. The

transition from childhood and elementary school to adolescence and secondary school is linked to shifting expectancies about the effects of substances from negative to more positive. It is generally assumed that the most effective method to prevent substance use in this stage is the social influence approach. The main emphasis of this approach is to make students aware of the various social pressures to use substances to be psychologically prepared to resist these influences.

Middle adolescence is the beginning of the separation and individuation from the family, with concurrent strivings for autonomy and independence. The relationships with parent's change. Increased involvement with the peer group leads to a rejection of parental values. Middle adolescents are increasingly capable of abstract thinking and organize complex thoughts about other people. Consequently, they develop a greater understanding of other's feelings and perspectives. Besides, there are marked changes in the way middle adolescents' value and weigh the relative costs and benefits of potentially risky behaviors. They become increasingly aware of the potential benefits of substance use and become less convinced of costs and risks. During adolescence, the brain undergoes considerable maturation. These changes are assumed to be responsible for great and rapid fluctuations in emotional states and increased sensitivity to rewarding outcomes. It is expected that substance use prevention is extremely difficult in this developmental stage, as middle adolescents are generally not open to adults' views. Peers are the primary source of reinforcement. Therefore, it is expected that the best results will be achieved through peer education.

Late adolescence includes the formation of identity, planning the future, and acquisition of the necessary skills to make a successful transition into adulthood. Late adolescents experience fundamental changes in their self-definition and identity. In late adolescence, relationships with parents usually improve, and the values of parents regain importance. Despite the diminished vulnerability to peer pressure and improvement of self-regulatory capabilities, late adolescence is characterized by the escalation of alcohol and drug use. For late adolescents, many social activities occur in drinking contexts and substance use becomes more and more normalized. It is expected that late adolescents benefit most from programs addressing their individuality and future orientation.

Schools are the focus of most attempts to develop effective approaches to drug abuse prevention. In addition to their traditional educational mission, schools often assume responsibility for addressing a variety of social and health problems, such as health education that targets tobacco, alcohol, and drug abuse, etc.

It is well established that substance use at a young age is associated with an elevated risk of drug abuse later in life, mental health problems, which may entail a substantial disease burden, followed by significant social and economic costs. Therefore, the need for effective prevention programs is very much desired.

As children spend a large part of their time at school, one important venue to reach large numbers of children and adolescents is through prevention programs that are offered at schools.

A large range of preventive programs has been launched that is based on proper school-based education and training program. These programs are also aligned with these developmental stages to prevent youngsters from using tobacco, alcohol, and drugs.

Model prevention programs- The creation of model prevention programs originates by recognizing that the drug problem is a reality that all students must face. For this reason, different models have been developed that give students, according to prevention levels, a series of skills and abilities that they need throughout their educational process that serve as tools to better coexist with others, avoid drug use, and confront the risks and influences in their environment that can alter their life plan. These skills contribute and stimulate healthy growth, and that all prevention programs should seek to strengthen them.

a. Life Skills Model: Seeks to strengthen psychosocial skills in children and adolescents that enable them to evolve in different contexts and situations that arise in the course of life.
b. Encouragement of commitment to a healthy lifestyle: promotes attitudes, habits, and values consistent with a healthy lifestyle.
c. Risk- and protective-factor specific model: seeks to strengthen protective factors associated with drug abuse and to diminish or avert risk factors connected principally with personal, family, cultural, and contextual circumstances.
d. Social competency model: builds up attitudes for acquitting oneself in different contexts and entails the acquisition of knowledge, aptitudes, and socio-affective skills.
e. Ecological model (preventive youth culture): incorporates cultural variables from the micro and macro levels.

These model prevention programs share the importance to combine clear and objective information on drugs and their consequences that attacks myths and tells children about the harm that substance abuse causes, to increase risk perception, and reduce the possibilities of initiation in drug

use. It is advisable to strengthen the development of these protective factors with attractive, specific programs that provide the educator with an instrument cum-work tool to reinforce their preventive role. Flexibility and ease of application are essential requirements for the viability of such models. The inclusion of the family is a core aspect of prevention. Close ties between family and school encourage awareness of and involvement in the development of children. Strengthening of personal and social skills, leading to the adoption of an active and responsible attitude.

GOVERNMENT POLICIES/ PROGRAMS AND LAWS

Drug abuse becomes a big problem in our country especially in these state Manipur, Mizoram, Nagaland, Himachal Pradesh, Punjab, Haryana, and Western Rajasthan. Punjab and Manipur are at the top of this list. At present, drug abuse is being seen as a complex problem because it is internally linked with other serious crimes such as organized crimes, human trafficking, and money laundering. In the last three decades (following the inception of the NDPS Act), the inistry of Social Justice and Empowerment has conducted two nation-wide drug surveys, published in 2004 and 2019. Drug addicts are using a substance that is easily available like cough syrups, pain killer ointments, glue, colophony, paints, gasoline, and cleaning fluids.

A government scheme for prevention of alcoholism and drug abuse-
The Government believes in addressing the problem of substance abuse in its totality. This includes creating awareness, early identification, treatment and rehabilitation, and sustained follow-up care. Further, the Government is of the view that substance abuse is a psychosocial medical problem that can be best addressed through community-based interventions. Hence, special emphasis has been given for involving and mobilizing the community. Under the Scheme for Prevention of Alcoholism and Substance (Drugs) Abuse, implemented by the Ministry of Social Justice and Empowerment, the non-governmental organizations have been entrusted with the responsibility for delivery of services and the Ministry bears substantial financial responsibility (90% of the prescribed grant amount).

The aim and objectives of the scheme for Prevention of Alcoholism and Substance Abuse are:

i. To support activities of non-governmental organizations, working in the areas of prevention of addiction and rehabilitation of addicts.

ii. To create awareness and educating the people about the ill effects of alcoholism and substance abuse on the individual, the family, and society at large.
iii. To develop culture-specific models for the prevention of addiction and treatment and rehabilitation of addicts.
iv. To evolve and provide a whole range of community-based services for the identification, motivation, detoxification, counseling, aftercare, and rehabilitation of addicts.
v. To promote community participation and public cooperation in the reduction of demand for dependence-producing substances.
vi. To promote collective initiatives and self-help endeavors among individuals and groups vulnerable to addiction and considered at risk.
vii. To establish appropriate linkages between voluntary agencies working in the field of addiction and government organizations.

The Scheme of Government provides financial support in the following areas-:

Awareness and Preventive Education, Drug Awareness and Counselling Centres, Treatmentcum- Rehabilitation Centres, Workplace Prevention Programmes, Deaddiction Camps, NGO Forum for Drug Abuse Prevention, Innovative Interventions to Strengthen Community Based Rehabilitation, Technical Exchange & Manpower Development, Surveys, Studies, Evaluation and Research,Any other activity considered suitable to meet the objectives of the Scheme Regional resource and training centers- Demand reduction means prevention, treatment, rehabilitation, and all other efforts that help end the craving and desire for drugs. Drug-related problems cannot be tackled by the use of force or prohibitive steps, legal or otherwise but by positive life-affirming measures on force or prohibitive measures, legal or otherwise, but must be a positive, life-affirming campaign too. People must be led, taught, convinced, and supported so that they can avoid the allure of drugs and draw emotional nourishment from wholesome activities. To that end, many people will also require material resources and reallife opportunities they never had before. The best long-term strategy for reducing demand lies with:

i. Creating more satisfying life conditions for as many people as possible, especially the disadvantaged;
ii. Educating everyone, especially the young, about the dangers of drugs, and just as importantly, teaching healthy alternatives to drug use;
iii. Providing effective rehabilitation for people who become addicted and helping them become productive citizens, positive role models, and active supporters of demand reduction efforts.

Keeping the above in view, it is obvious that any effort in this direction requires a sustained, committed, and long-term intervention with a flexible and innovative approach. The Government of India has been, for the last 15 years, promoting and supporting culture-specific, need-oriented, and localized community-based interventions and initiatives through Non-Governmental Organizations (NGOs). It is, however, realized that effective and appropriate service delivery through the NGOs, while on the one hand requires continuous and regular professional development of the service providers in inculcating skills about newer strategies for dealing with addiction, on the other, documentation of the implementation of the programs, monitoring of the changes in the profile of the drug abuse scenario and technical support to be able to respond to the changing situations, needs to be taken care of. In a country like India, with diverse physical, social and cultural features, it is necessary that the adopted strategy takes into account the regional needs and local field realities in undertaking activities related to advocacy, research, monitoring, and training. To meet the regional requirements and aspirations, the Ministry of Social Justice and Empowerment, in collaboration with UNODC and ILO, has evolved a functional structural mechanism.

National Centre for Drug Abuse Prevention (NCDAP)- The Ministry has established National Centre for Drug Abuse Prevention (NCDAP) at New Delhi, to serve as an apex body for training, research, and documentation in the field of alcohol and drug demand reduction. It was set up in NISD in 1998 with the following objectives:

i. Raising the competency standards of the functionaries/ personnel working in Deaddiction centers and other related sectors.
ii. Invigorating efforts for preventing or minimizing any anticipated or consequent harm related to Drug Abuse in clients and the community.
iii. Standardization of care in Drug Abuse Prevention.
iv. Updating information and creating a database on the extent and pattern of Drug Abuse and its various interventions at local, regional, national, and international levels.
v. Developing standards of monitoring systems of various interventions.

The Regional Resource and Training Centres- As the name suggests, have been established essentially for the devolution of the mandate of NCDAP at the regional level. They would, as representatives their region undertake the following activities:
a) Training of service providers.
b) Documentation of programs/results.
c) Advocacy, research, and monitoring.
d) Technical support to NGOs, CBOs, and Enterprises.

e) Strengthening the rehabilitation of the addicts.

The NGOs, all over the country, working in the field of alcohol and drug demand reduction are expected to liaison with the National Centre of Drug Abuse Prevention, NISD, New Delhi, and the respective Regional Resource and Training Centre for required technical assistance and manpower development.

Statutory Provisions- There are various statutory provisions to prevent drug abuse. The main laws or acts enacted are –

A. The Narcotic Drugs and Psychotropic Substances Act, 1985 (Amended in 2001 and 2014)
B. Prevention of Illicit Traffic in Narcotic Drugs and Psychotropic Substances Act, 1988.
C. Drugs and Cosmetics Act, 1940

A. **Narcotic Drugs and Psychotropic Substances Act (NDPS)-1985**- This is the main legislation (law) on narcotic drugs and psychotropic substances in India and in short, called as NDPS Act, 1985. NDPS act repealed or cancels previous central acts namely, the Opium Act, 1857, the Opium Act, 1878, and the Dangerous Drugs Act, 193032, for the better enforcement of the law against drug abuse. The major deficiencies or drawbacks of these previous acts were either due to insufficient penalties deterrent to meet the challenge of well-organized gangs of smugglers or no provision to enable exercise of control over psychotropic substances. Therefore, a comprehensive law on narcotic drugs and psychotropic substances was enacted as the NDPS Act 1985. The main objectives of the NDPS Act, 1985 are:

i. To provide deterrent punishments to drug offenders.
ii. Invest central agencies with powers of investigation of drug offenses.
iii. To take care of obligations arising under certain international conventions on drugs to which India is a party
iv. To control psychotropic substances covered under the Convention on Psychotropic Substances

The NDPS Act, 1985 sets out the statutory framework for drug law enforcement in India. The main features of this act are as follow:

a) The cultivation, production, manufacture, possession, sale, purchase, transportation, warehousing, consumption, inter-state movement, trans-shipment and import and export of narcotic drugs and psychotropic substances is prohibited except for medical or scientific purposes and under

the terms and conditions of any license, permit or authorization given by the Government.

b) Prohibition of certain activities relating to property derived from offenses. For example, conversion/transfer of property knowing that such property is derived from any offense under the Act; concealment of the true nature/source/location of such property knowingly acquire, possess or use any such property.
c) The Central Government is empowered to regulate the cultivation production, manufacture, import, export, sale, consumption, and use of narcotic drugs and psychotropic substances.
d) State Governments are empowered to permit and regulate the possession and inter-state movement of opium, poppy straw, the manufacture of medicinal opium, and the cultivation of cannabis excluding hashish.
e) All persons in India are prohibited from engaging in or controlling any trade whereby narcotic drugs or psychotropic substances are obtained outside India and supplied to any person outside India except with the previous authorization of the Central Government and subject to such conditions as may be imposed by the Central Government93.
f) The Central Government is empowered to declare any substance, based on an assessment of its likely use in the manufacture of narcotics drugs and psychotropic substances as a "controlled substance.
g) Assets derived from drug trafficking are liable to forfeiture and the sale proceeds to be credited to the National Fund for Control of Drug Abuse.
h) Both the Central Government and State Governments are empowered to appoint officers for the Act.
i) Offenses under the Act to be cognizable and non-bailable.

The NDPS Act is a comprehensive code not only for the control and regulation of narcotics drugs and psychotropic substances but also for the investigation and forfeiture of drug-related assets. Centre Government, as well as State government, have been empowered to enforce the provisions of the Act. The State enforcement agencies including the Police, Excise, and Drugs Control Department are also involved in the administration of the NDPS Act.

Offenses and Penalties-Punishment for offenses are essentially related to violations of the various prohibitions imposed under the act on the cultivation, production manufacture, distribution, sale, import, and export of narcotic drugs and psychotropic substances. All these offenses are triable by Special Courts, and very stringent punishments are as follows-

i. Ranging from 6 months minimum to 30 years' imprisonment depending upon the nature of the offenses.
ii. Imprisonment range from 10-20 years for first offenses to 15-30 years for any subsequent offenses together with fines up to 2 lakhs; provided that the court may have reasons to be recorded in the judgments, impose a fine exceeding 2 lakhs rupees; and the punishment shall be based on the "quantity involved".
iii. The sentencing structure underwent a drastic change with the enactment of the Amendment Act, in 2001.
iv. The Act introduced the concept of "commercial quantity" about narcotic drugs or psychotropic substances.
v. Under this rationalized sentencing structure, the punishment would vary depending on whether the quantity of offending material was "small quantity", "commercial quantity" or "something in between"

Punishment details based on the quantity of drugs

S.No	Category	Drug and quantity	Punishment
1.	Small Quantity	Heroin (5gram) Opium (25gram) Morphine (5 gram) Cannabis (Ganja)(1000 (gram) Cannabis resin Charas) (100 gram) Coca leaf (100 gram) Cocaine (2 gram) Methadone(2 gram) Amphetamine (2 gram) LSD (0.002 gram)	Maximum 1 year Imprisonment or Fine of upto Rs 10000 or Both
2.	Large quantity	Heroin (250 Gram) Opium (2.5 kg) Morphine (250 gram) Cannabis (Ganja)(20 kg) Cannabis resin Charas) (1 kg) Coca leaf (2 kg) Cocaine (100 gram) Methadone(50 gram) Amphetamine (50 gram) LSD (0.1gran)	Rigorous Imprisonment minimum 10 year and maximum 20 years and Fine of Rs 1-2 lakh

3.	Intermediate	Quantity greater than small quantity but lesser than commercial quantity	Rigorous Imprisonment that may extend to 10 Yrs and Fine extend to Rs 1 lakh

NDPS Amendment Act 2001- NDPS Act was further amended in October 2001. The most significant amendments include: changing the law to allow for sentencing to be based on the size of the drug seizure and formally authorizing controlled deliveries inside and outside of India. Before these changes, individuals found with small amounts of illicit drugs were subject to the same penalties as large-scale drug traffickers. The Amendment in 2001 provided for the rationalization of sentence structure. However, bail provisions were liberalized.

NDPS Amendment Act 2014- In early 2014, the NDPS Act was amended for the third time and the new provisions came into force on 1 May 2014. The main features include:

i. Creation of a new category of "essential narcotic drugs", which the central government can specify and regulate.
ii. Including the terms "management" of drug dependence and "recognition and approval" of treatment centers, thus allowing for the establishment of legally binding treatment standards and evidence-based medical interventions
iii. Making the death penalty discretionary for a subsequent offense involving a certain quantity of drugs under section 31A. The court will have the alternative to impose imprisonment for 30 years under this section.
iv. Enhanced punishment for small quantity offenses from a maximum of six months to one-year imprisonment.
v. Allowing private sector involvement in the processing of opium and concentrated poppy straw.
vi. Raising the rank of officers authorized to conduct search and arrest license holders for alleged NDPS violations.
vii. More elaborative provisions for forfeiture of property of persons arraigned on charges of drug trafficking.

2. Prevention of Illicit Traffic in Narcotic Drugs and Psychotropic Substances Act 1988- Illicit traffic in narcotic drugs and psychotropic substances poses a serious threat to the health and welfare of the people; and the activities of persons engaged in such illicit traffic have a deleterious effect on the national economy. Having regard to the persons by whom and how such activities are organized and carried on; and having regard to the

fact that in certain "areas which are highly vulnerable to the illicit traffic in narcotic drugs and psychotropic substances", such activities of considerable magnitude are clandestinely organized and carried on, the Parliament found it necessary for the effective prevention of such activities and to provide for the detention of persons concerned in any manner therewith.

With these objectives, the Prevention of Illicit Traffic in Narcotic Drugs and Psychotropic Substances Act,1988 was enacted. For this Act "area highly vulnerable to such illicit traffic" means

i. The Indian customs waters;
ii. The customs airports;
iii. The metropolitan cities of Bombay, Calcutta, Delhi, Madras, and the city of Varanasi;
iv. The inland area one hundred kilometers in width from the coast of India falling within the territories of the States of Andhra Pradesh, Goa, Gujarat, Karnataka, Kerala, Maharashtra, Orissa, Tamil Nadu and West Bengal and the Union territories of Daman and Diu and Pondicherry
v. The inland area one hundred kilometers in width from—
    a) The India-Pakistan border in the States of Gujarat, Punjab, and Rajasthan;
    b) The India-Nepal border in the States of Bihar, Sikkim, Uttar
    c) Pradesh and West Bengal;
    d) The India-Burma border in the States of Arunachal Pradesh,
    e) Manipur, Mizoram, and Nagaland; Such other area or customs station, as the Central Government may, having regard to the vulnerability of such area or customs station, as the case of being, to illicit traffic, by notification in the Official Gazette, specify in the behalf

The Act empowers the Central Government and State Governments to make an order directing any person including a foreigner be detained to prevent him from engaging in illicit traffic in narcotic drugs and psychotropic substances. Thus, the Act provides for preventive detention. The Act further states that ' no detention order shall be invalid or inoperative merely by reason – (a) that the person to be detained thereunder is outside the limits of the territorial jurisdiction of the Government or the officer making the order of detention; or (b) that the place of detention of such person is outside the said limits. Moreover, if the person absconds or conceals himself, he shall be punishable with imprisonment for a term which may extend to one year, or with fine or, with both; and the offense shall be cognizable.

3. **Drug and Cosmetic Act 1940**- Drugs and Cosmetics Act was adopted in 1940 to regulate the manufacturing and sale of medicinal drugs including cannabis and opium. With the adoption of the Indian Constitution in 1950, Drug Laws got a completely new dimension under Article 47 which specifically states that the State must bring about prohibition of the consumption of drugs except for medicinal purposes. Even under the Directive Principles of State Policy provisions for drug policies are justified. The Constitution also placed matters of "Drugs and poisons" in the concurrent list, allowing both centers and states to legislate. The Drugs and Cosmetics Act, 1940, continues to be in force in respect of formulations that contain narcotic drugs and psychotropic substances.

4. **Juvenile Justice (Care and Protection of Children) Act, 2000**- To prevent the illegal supply of narcotic drugs to persons below the age of eighteen, Parliament has enacted this legislation. The penal provision is extracted hereunder: Whoever gives, or causes to be given, to any juvenile or the child any intoxicating liquor in a public place or any narcotic drug or psychotropic substance except upon the order of duly qualified medical practitioner or case of sickness, shall be punishable with imprisonment for a term which may extend to three years and shall also be liable to fine. The offense shall be cognizable. Section 2 (d) includes in the definition of a child in need of care and protection' children vulnerable to or likely to be inducted into drug abuse or drug trafficking.

Law enforcement agencies- To implement or enforce these statutory provisions or laws effectively, the government has set up law enforcement agencies. The various law enforcing agencies includes

a) **Narcotics Control Division (NCD)-** The Narcotics Control Bureau is the apex coordinating agency. It also functions as an enforcement agency The NCD which was created as the nodal agency for coordination of the multiple bodies including intelligence collection, dissemination, enforcement, and coordination with various Central and State enforcement organizations and international, regional, and foreign enforcement agencies. The Bureau's responsibilities include administrative coordination with different Union Ministries, State Government Departments, and the various Central and State Law Enforcement agencies for effective implementation of the various regulatory, prohibitory penal, and administrative provisions of the NDPS Act. The Bureau also acts as the nodal agency for intelligence and enforcement organizations on the one side; and the international, regional, and friendly-foreign enforcement agencies on the other side. The Bureau, subject to the supervision and control of the Central

Government, is to exercise the powers and functions of the Central Government for taking measures concerning:

i. Co-ordination of actions by various offices, State Governments, and other authorities under the NDPS Act,1985.
ii. Implementation of the obligation in respect of countermeasures against illicit traffic under the various international conventions and protocols that are in force at present or which may be ratified or acceded to by India in the future.
iii. Assistance to concerned authorities in foreign countries and concerned international organizations to facilitate coordination and universal action for prevention and suppression of illicit traffic in narcotic drugs and psychotropic substances.
iv. Co-ordination of actions taken by the other concerned Ministries, Departments, and Organizations in respect of matters relating to drug abuse.
v. Identification, treatment, education, aftercare, rehabilitation and social reintegration of addicts

b) Central Bureau of Narcotics- It is headed by the Narcotic Commissioner and the office is located in Gwalior. CBN is responsible for all aspects of the opium industry and preventing illicit precursor chemical trafficking. CBN has been primarily given the responsibility of licensing and supervising opium cultivation and thereafter, procuring opium from the cultivators including anti-narcotic operations all over the country to checks the spread of the contraband as well as the cultivation of drugs. The responsibilities of CBN include

i. Supervision over licit cultivation of opium poppy in India.
ii. Investigation of cases under the NDPS Act, 1985, and filing of a complaint in the Court.
iii. Action for tracing and freezing of illegally acquired property.
iv. Issue of licenses for the manufacture of synthetic narcotic drugs.
v. Issuance of Export Authorizations/Import Certificate for export/import of Narcotic Drugs and Psychotropic Substances.
vi. Interaction with the International Narcotics Control Board, Vienna, and the Competent Authorities of other countries to verify the genuineness of the transaction before authorizing the shipments.

c) Other Agencies

i. **Directorate of Revenue Intelligence**- is a part of the Ministry of Finance and is responsible for information on the smuggling of goods, including drugs into, or out of, India. Other law enforcement agencies

with counter drug responsibilities are the Central Bureau of Investigation, the Customs Commission, and the Border Security Force. The Customs Commission has a wide variety of drug law enforcement tasks and falls under the Ministry of Finance's Central Board of Excise and Customs. The Border Security Force, under the Home Ministry, is a paramilitary force that controls India's land borders and frequently interdicts drug shipments.

ii) **Department of Revenue**- is the nodal department responsible for the administration of the Narcotic Drugs and Psychotropic Substances Act, 1986 and implementation of international conventions, treaties, bilateral agreements, and MOUs6. Narcotics Control Division in the Department of Revenue facilitates and co-ordinates the functioning of the Narcotics Control Bureau (NCB), Central Bureau of Narcotics (CBN), and the Chief Controller of Factories (CCF).

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